

PRODUCT LITERATURE

METACIN TABLET 650 MG

Each tablet contains

Paracetamol 650 mg

Preservative

Sodium Benzoate 0.1%w/w

Description

White coloured odourless, oval shaped tablet with 'TPI' embossed on one side and plain on the other side.

Pharmacodynamics Properties

Paracetamol is an antipyretic analgesic. The mechanism of action is probably similar to that of aspirin and dependent on the inhibition of prostaglandin synthesis. This inhibition appears, however to be on a selective basis.

Pharmacokinetics Properties

Paracetamol is rapidly and almost completely absorbed from the gastrointestinal tract. Concentration in plasma reaches a peak in 30 to 60 minutes. Plasma half-life is 1 - 4 hours. Paracetamol is relatively uniformly distributed throughout most body fluids.

Binding of the drug to plasma proteins is variable; 20 to 30% may be bound at the concentrations encountered during acute intoxication. Following therapeutic doses 90-100% of the drug may be recovered in the urine within the first day. However, practically no paracetamol is excreted unchanged and the bulk is excreted after hepatic conjugation. Excretion is almost exclusively renal, in the form of conjugated metabolites.

Indication

Analgesic and antipyretic. Indicated for the relief of mild to moderate pain and to reduce fever.

Route of administration :

Oral.

Recommended Dose

Adult and children above 12 years: 1 tablet every 4 to 6 hours (not more than 6 tablets in one day).

Contraindications

Hypersensitivity to paracetamol or any of the other constituents.

Warning and Precautions

This preparation contains PARACETAMOL.

Do not take any other paracetamol containing medicines at the same time.

• **Allergy alert:** Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away

- Contains paracetamol. Do not use with any other paracetamol-containing products.
- Underlying liver disease increases the risk of paracetamol related liver damage. Patients who have been diagnosed with liver or kidney impairment must seek medical advice before taking this medication. Do not exceed the stated dose.
- Patients should be advised to consult their doctor if their headaches become persistent.
- Caution should be exercised in patients with glutathione depleted states, as the use of paracetamol may increase the risk of metabolic acidosis.
- Use with caution in patients with glutathione depletion due to metabolic deficiencies.
- If symptoms persist, medical advice must be sought.
- Keep out of the sight and reach of children.

Interactions with Other Medicaments

The speed of absorption of paracetamol may be increased by metoclopramide or domperidone and absorption reduced by colestyramine. The anticoagulant effect of warfarin and other coumarins may be enhanced by prolonged regular daily use of paracetamol with increased risk of bleeding; occasional doses have no significant effect.

Pregnancy and Lactation

Epidemiological studies on neurodevelopment in children exposed to paracetamol in utero show inconclusive results. If clinically needed, paracetamol can be used during pregnancy if clinically needed, however, as with any medicine it should be used at the lowest effective dose for the shortest possible time.

Paracetamol is excreted in breast milk, but not in a clinically significant amount in recommended dosages. Available published data do not contraindicate breast feeding.

Side Effect

Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.

Symptoms and Treatment of Overdose

Liver damage is possible in adults who have taken 10 g or more of paracetamol. Ingestion of 5 g or more of paracetamol may lead to liver damage if the patient has risk factors (see below).

Risk Factors

If the patient

a. is on long term treatment with carbamazepine, phenobarbitone, phenytoin, primidone, rifampicin, St John's Wort or other drugs that induce liver enzymes.

Or

b. regularly consumes ethanol in excess of recommended amounts.

Or

c. is likely to be glutathione depleted e.g. eating disorders, cystic fibrosis, HIV infection, starvation, cachexia.

Symptoms

Symptoms of paracetamol overdose in the first 24 hours are pallor, nausea, vomiting, anorexia and abdominal pain. Liver damage may become apparent 12 to 48 hours after ingestion. Abnormalities of glucose metabolism and metabolic acidosis may occur. In severe poisoning, hepatic failure may progress to encephalopathy, haemorrhage, hypoglycaemia, cerebral oedema and death. Acute renal failure with acute tubular necrosis, strongly suggested by loin pain, haematuria and proteinuria, may develop even in the absence of severe liver damage. Cardiac arrhythmias and pancreatitis have been reported.

Management

Immediate treatment is essential in the management of paracetamol overdose. Despite a lack of significant early symptoms, patients should be referred to hospital urgently for immediate medical attention. Symptoms may be limited to nausea or vomiting and may not reflect the severity of overdose or the risk of organ damage. Management should be in accordance with established treatment guidelines, see BNF overdose section.

Treatment with activated charcoal should be considered if the overdose has been taken within 1 hour. Plasma paracetamol concentration should be measured at 4 hours or later after ingestion (earlier concentrations are unreliable). Treatment with N-acetylcysteine may be used up to 24 hours after ingestion of paracetamol, however, the maximum protective effect is obtained up to 8 hours post-ingestion. The effectiveness of the antidote declines sharply after this time. If required the patient should be given intravenous N-acetylcysteine, in line with the established dosage schedule. If vomiting is not a problem, oral methionine may be a suitable alternative for remote areas, outside hospital. Management of patients who present with serious hepatic dysfunction beyond 24h from ingestion should be discussed with the NPIS or a liver unit.

Packing

Packed in blister-packs of 10 x 10's / Box

Storage Conditions

Store below 30°C in a dry place, protected from direct light. Keep away from reach of children.

Expiry: 3 years from date of manufacture

Name and Address of Manufacturer:

TERAPUTICS SDN. BHD. (590500-W)
Lot 10 & 11, PERDA Industrial Park,
Lorong IKS Simpang Ampat B,
14100 Simpang Ampat, S.P.S.,
Pulau Pinang, Malaysia

Name and Address of Product Registration Holder/ Distributor:

ZONTRON PHARMACEUTICALS SDN.BHD. (445695-T)
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Malaysian Drug Registration No. :

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